



GUIDELINE

Consent

Scope (Staff):	Clinical Staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

Contents

Aim	2
Risk.....	2
Background.....	2
Definitions	3
Principles	4
Capacity to Consent.....	5
Assessing Capacity in Children	5
Children with Capacity	6
Children without Capacity	6
Wishes of Child and Parent/Guardian.....	6
Valid Consent	7
Reviewing Consent.....	7
Circumstances where Consent is Required.....	8
Consent to Assessment.....	8
Mental Health Assessments.....	8
Physical Health Assessments	8
Consent to Treatment.....	8
Explaining the Proposed Treatment.....	8

Time to Consider the Proposed Treatment	9
Medication.....	9
Clozapine.....	9
Off-label Medication	10
Electroconvulsive Therapy	10
Voluntary Patients.....	10
Involuntary Patients	10
Children Under 14.....	11
Consent to Exchange Information	11
Duty of Confidentiality	11
Consent for the Release of Information.....	11
Implied Consent for the Release of Information	12
Circumstances Requiring Disclosure of Health Information.....	12
Circumstances where Consent is not Required.....	12
Emergency Psychiatric Treatment.....	12
Involuntary Mental Health Patients or the Mentally Impaired Accused.....	13
Psychiatric Treatment	13
Urgent Non-Psychiatric Treatment.....	13
Documentation.....	13
Compliance.....	14
Appendix 1: CAMHS Consent Documentation	17

Aim

To provide guidance to clinical staff to meet their legal obligations in seeking consent.

Risk

Legal claims including trespass to the person, negligence or inadequate consumer consultation.

Background

This policy aligns with the [Mental Health Act 2014 \(WA\)](#) (MHA), [Clinicians' Practice Guide to the Mental Health Act 2014](#), WA Health [Consent to Treatment](#) policy, WA Health [Patient Confidentiality](#) fact sheet, and the CAHS [Consent to Treatment](#) policy.

Definitions

Capacity: A person has capacity if they can make a decision about a matter pertaining to themselves and are able to:

- a. Understand any information or advice about the decision;
- b. Understand the matters involved in the decision;
- c. Understand the effect of the decision;
- d. Weigh up the factors referred to in paragraphs (a), (b) and (c) when making the decision; and
- e. Communicate the decision in some way (MHA s 15(1)).

For the purposes of the MHA, children are presumed to not have capacity unless determined otherwise (MHA s 14(1)).

Adults are presumed to have capacity unless determined otherwise (MHA s 13(1)).

A person's capacity to consent can change depending on the circumstances. Capacity must be assessed every time a person seeks consent.

Carer: A person who provides care or assistance to a person who meets the criteria in s 5(1) of the [Carers Recognition Act 2004](#) (WA). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (MHA s 281(2)(b)).

Child: A person under 18 years of age (MHA s 4).

Emergency psychiatric treatment: Emergency psychiatric treatment is treatment that needs to be provided to a person to save that person's life or to prevent that person from behaving in a way that is likely to result in serious physical injury to themselves or another person (MHA s 202(1)).

Explicit (express) consent: Explicit consent is consent that is clearly and unmistakeably stated.

Health information: Information (or an opinion) that is also personal information about the health or disability of an individual, an individual's expressed wishes regarding their healthcare, health services provided to an individual, or any personal information collected for the purpose of providing a health service (this would include patient or client medical or health care records).¹³

Implied consent: Implied consent is consent which is not explicitly granted by a person, but rather implicitly granted through the actions of a person and the facts and circumstances of a situation.

Involuntary patient: An involuntary patient is a person who is under an involuntary treatment order (MHA s 21(1)). An involuntary treatment order is an inpatient treatment order or a community treatment order (MHA s 21(2)). For more information regarding involuntary treatment orders please refer to the MHA (MHA ss 22(1), 23(1)).

Mature minor: A child who can fully comprehend the nature, consequences and risks of a proposed action, irrespective of the presence or absence of parental consent (*Gillick v West Norfolk and Wisbech Area Health Authority* [1986] AC 112).

Mentally impaired accused: A person who has been charged with an offence and is under a current custody order that they have not been discharged from ([Criminal Law \(Mentally Impaired Accused\) Act 1996](#) (WA) s 23).

Nominated person: The person nominated under section 273(1) of the MHA. The role of a nominated person is to assist the child by making sure that the child's rights and interests are upheld. A nominated person could, for example, be a friend or relative of the child or a child's guardian. The person nominated must be an adult over the age of 18. The formal requirements for a valid nomination are outlined in section 275 of the MHA.

Parent/Guardian: The person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4, [Family Law Act 1975](#) (Cth) (FLA) s 61B and [Family Court Act 1997](#) (WA) (FCA) s 68). For the purposes of this guideline, this also includes any legally appointed guardian of the child (FCA s 71). Changes to relationships such as parental separation, divorce or remarriage do not alter parental responsibility, unless there is a specific court order changing parental responsibility (FLA s 61C(2), FCA s 69(2)).

Treatment: The provision of a psychiatric, medical, psychological or psychosocial intervention intended (whether alone or in combination with one or more other therapeutic interventions) to alleviate or prevent the deterioration of a mental illness or a condition that is a consequence of a mental illness, and does not include bodily restraint, seclusion or sterilisation (MHA s 4).

Principles

- Clinicians must consider what is in the best interests of the child (MHA s 299).
- Consent is valid if it is voluntary, informed, current and is given by a child who has capacity or their parent/guardian.¹²
- For the purposes of the MHA, children are presumed to not have capacity unless determined otherwise (MHA s 14(1)).
- Explicit consent should always be sought by health service providers where possible.
- Silence is not tantamount to consent.
- Failure to offer resistance by the child or their parent/guardian does not constitute consent (MHA s 16(2)).
- Gaining informed consent to treatment is an essential component of recovery-oriented practice.

- The process of gaining informed consent must take into account cultural and social considerations, including considerations around Aboriginality, ethnicity, disability, religion, spirituality or belief.
- Additional support must be considered for children and families who may require language services, including children who are deaf, from a culturally and linguistically diverse background and those who are Aboriginal Australians, to ensure valid consent. See the CAHS [Consent to Treatment](#) policy for additional information.
- The most senior health professional is responsible for ensuring that informed consent has been given and that it is documented. This responsibility can be delegated. Delegation must be documented in the medical record.

Capacity to Consent

Decision making capacity must be assessed as part of the consent process.

Assessing Capacity in Children

For the purposes of the MHA, children are presumed to not have decision making capacity unless an assessment of the child determines otherwise (MHA s 14(1)). Mature minors are children that have been assessed as having decision making capacity.

Assessing capacity in children must be made through consideration of the criteria in section 15(1) of the MHA. These criteria are outlined in the [definitions](#) section of this guideline.

Capacity must be assessed using a communication style that the child can understand and respond to (MHA s 9(2)). This may require engaging an interpreter, using contextually and developmentally appropriate language or employing tools such as speech therapy cards to assist with non-verbal communication.

When determining the decision-making capacity of a child the assessing clinician must consider:

- The age of the child (particularly in relation to the child's cognitive ability to understand the current issues and circumstances);
- The child's maturity and degree of autonomy;
- The capacity of the child to understand the nature and consequences of the decision in question;
- The type and sensitivity of the information discussed; and
- The complexity and nature of the treatment (for example elective, therapeutic or emergency).

Previous or suspected diagnosis of a mental illness does not, on its own, equate to an absence of decision-making capacity. Mental illness may be a relevant factor when determining capacity, but it must not be the only factor.

For further guidance on determining a child's capacity to consent, please refer to the *Assessing Capacity to Consent in Minors: A Clinical Handbook*.

Children with Capacity

Where a child is determined to have capacity, their capacity to consent must be clearly documented in their medical record by the assessing clinician.

Where a child is deemed to have capacity, the assessing clinician must ask whether the child consents to:

- Information being shared with their parent/guardian, carer or nominated person; and
- The involvement of their parent/guardian, carer or nominated person in decisions pertaining to their care.

Where a child with capacity elects not to have their parent/guardian, carer or nominated person involved in their care, their reasons for this decision must be clearly documented in the medical record.

For further information please refer to the CAMHS [Clinical Assessment](#) policy.

Children without Capacity

Where a child does not have capacity, the child's parent/guardian may make decisions on the child's behalf (MHA ss 14(2), 302(3)). This does not apply to children who are in the care of the CEO of the Department of Communities.

Adults are presumed to have decision making capacity. There may be circumstances however, where the child's parent/guardian does not have the capacity to consent on the child's behalf. Assessing capacity in adults is the same as assessing capacity in children. An assessment must be made through consideration of the criteria in section 15(1) of the MHA. These criteria are outlined in the [definitions](#) section of this guideline.

Where the parent/guardian does not have the capacity to consent on behalf of the child or where the parents/guardians cannot agree on whether to give consent, the CAHS Mediation and Legal branch must be contacted. This does not apply to [circumstances where consent is not required](#).

Wishes of Child and Parent/Guardian

Irrespective of whether the child is deemed to have capacity or not, it is best practice for clinicians to:

- Attempt gain the child's consent; and
- Involve the child as much as possible in decisions made on their behalf.

In any clinical decision making, particularly when performing a function under the MHA, consideration must be given to both the wishes of the child and the wishes of their parent/guardian (MHA ss 300, 301).

Valid Consent

The key elements of valid consent are:

- The consent must be given freely and voluntarily;
- The individual giving consent must be adequately informed;
- The consent must be current; and
- The individual giving consent must have the capacity to understand, provide and communicate their consent.

Any communication pertaining to consent must be in a form that the child or their parent/guardian can understand (MHA s 9(2)). This may involve engaging an interpreter to assist the clinician with communicating with the child or their parent/guardian.

To ensure that the person giving consent is adequately informed, it is best practice to ask the person to repeat their understanding of the information discussed. This creates an opportunity for the clinician to:

- Confirm that the information discussed was correctly understood; and
- Correct any information that was misconstrued.

Children and families must be made aware that their consent can be withdrawn at any time.

Consent is valid until it is withdrawn or the circumstances in which the consent was given have changed. This may include a change in the child's condition, development of new treatment options or progression of a disease.¹² For further information please refer to the [reviewing consent](#) section of this guideline.

Consent or refusal to consent must be documented in the medical record.

Reviewing Consent

Consent must be reviewed in the following circumstances:

- A change in the underlying condition being treated;
- Significant changes to the treatment consented to (such as a change to the modality of treatment or the therapeutic model);
- Commencement of a new treatment;
- Where new risks are identified in relation to the treatment consented to;
- Where new alternative treatments become available that may better treat the child's illness or condition (this shall be determined on a case by case basis, at the discretion of the treating clinician and in consultation with their multidisciplinary team);
- A change in status under the MHA; or

- A change in guardianship or parental responsibility for the child.

Circumstances where Consent is Required

Consent to Assessment

Mental Health Assessments

Clinicians must obtain consent prior to conducting a mental health assessment. This consent must be recorded on the CAMHS [Initial Assessment](#) form.

Physical Health Assessments

Medical practitioners must obtain consent prior to conducting a physical health assessment. This consent must be recorded on the CAMHS [Physical Examination](#) form.

Consent to Treatment

Consent to treatment must be explicit. Consent to treatment may cover all treatments under, or within, a course of treatment as per the CAHS [Consent to Treatment](#) policy. The child and their parent/guardian must be advised that their consent can be withdrawn at any time.

When consenting to treatment, the person giving consent must demonstrate that they can:

- Understand the proposed treatment;
- Understand the matters involved in making the treatment decision;
- Understand the effect of the treatment decision;
- Consider the above factors when making the treatment decision; and
- Communicate the treatment decision (MHA s 18).

Explaining the Proposed Treatment

The person giving consent to treatment must be provided with a clear explanation of the proposed treatment (MHA s 19(1)). A clear explanation of the proposed treatment must:

- Contain sufficient information to enable the person to make a balanced judgement about the proposed treatment;
- Identify and explain alternative treatment options; and
- Warn of any risks inherent in the treatment proposed (MHA s 19(1)).

Sufficient information must be provided using linguistically, developmentally and contextually appropriate language for the child or their parent/guardian (MHA s 9(2)). Sufficient information includes, but is not limited to, the following:

- The initial formulation;

- The proposed treatment and recovery plan;
- Any evidence-based alternatives to the proposed treatment plan;
- Expected outcomes with or without treatment;
- Potential risks of the proposed treatment;
- Consequences of delaying treatment;
- Consequences of withdrawal from the proposed treatment at a future date;
- Any short term or long-term side effects of the proposed treatment;
- Any follow-up treatments that may be needed; and
- Any other information that a reasonable person would likely consider significant in the circumstances (MHA s 19(2)).

Time to Consider the Proposed Treatment

The person giving consent must be allowed sufficient time to consider the proposed treatment before making a decision.

Sufficient time for consideration must allow for:

- The person giving consent to consider the matters involved in the treatment decision;
- A reasonable opportunity to discuss those matters with the health professional proposing the treatment in question; and
- A reasonable opportunity to obtain any other advice or assistance in relation to the treatment decision (MHA s 20).

Consent to treatment must be clearly documented in the medical record.

For more information regarding valid consent to treatment, please refer to [Appendix 2](#).

Medication

Psychiatrists must obtain explicit consent from the child (where the child has capacity) or their parent/guardian (where the child does not have capacity) prior to prescribing medication. This consent must be recorded on the [Consent for Medication Use](#) form (MR505.00/MR859.01).

For further information regarding the prescription of medication, please see the CAMHS [Psychotropic Medication – Monitoring Adverse Physical Health Effects](#) policy.

Clozapine

Clozapine requires explicit written informed consent from the child or their parent/guardian. The psychiatrist seeking to initiate the drug must provide all relevant information to the child or their parent/guardian, including the benefits and risks specific to the child who may receive the medication. In addition to explicit consent, initiation of Clozapine also requires approval from the CAHS Drugs and Therapeutics

Committee. When Clozapine is initiated, the Clozapine Patient Management System (CPMS) protocol must be adhered to. The CPMS is currently provided by [ClopineCentral](#).

For more information regarding the prescribing and approval requirements for Clozapine, please refer to the CAHS [Clozapine](#) monograph.

Off-label Medication

Where the proposed medication prescription is off-label, the psychiatrist must clearly explain the meaning of off-label as outlined in the [Consent for Medication Use](#) form. After this explanation has been provided, the consent to use off-label medications must be documented on the form.

Where the child has been prescribed off-label medication by a non CAMHS clinician, a CAMHS clinician must attempt to contact the non CAMHS prescriber to obtain a handover regarding consent for the off-label medication. This interaction must be documented in the medical record.

Where the CAMHS clinician is unable to make contact with the non CAMHS prescriber, their attempts to make contact must be documented in the medical record.

The CAMHS clinician must inform the child and their parent/guardian of the off-label nature of the medication and encourage them to discuss this with the prescriber. This interaction must be documented in the medical record.

Electroconvulsive Therapy

CAMHS may organise for ECT to be administered by another hospital but do not themselves administer this treatment.

Voluntary Patients

For voluntary patients, electroconvulsive therapy (ECT) requires explicit written informed consent from the child or their parent/guardian (MHA s 195(2)(a)). The psychiatrist seeking to initiate the treatment must provide all relevant information to the child and their parent/guardian, including the benefits and risks specific to the child who may receive the treatment. The child or their parent/guardian must be informed that their consent is for the entire course of treatment and that they may withdraw their consent at any time.¹² Their consent must be documented in the medical record.

In addition to consent from the child or their parent/guardian, the Mental Health Tribunal must also approve this treatment (MHA s 195(2)(b)).

Involuntary Patients

For involuntary patients or the mentally impaired accused, ECT can be performed without explicit written informed consent from the child or their parent/guardian (MHA s 196(2)). Approval from the Mental Health Tribunal is still required before ECT can be administered in these circumstances (MHA s 196(2)(a)).

Children Under 14

ECT cannot be administered to a child under the age of 14, irrespective of whether explicit written informed consent to this treatment has been given by the child or their parent/guardian (MHA s 194).

For more guidance on the required approvals for ECT please refer to the following:

- [MHA](#) (Part 14 Division 1, Part 21 Division 6)
- [Clinicians' Practice Guide to the Mental Health Act 2014](#) (Section 9.8)
- [Mental Health Tribunal Practice Manual](#) (Part 3 Chapter 6)
- [The E.C.T Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2006](#)

Consent to Exchange Information

Duty of Confidentiality

Health professionals have a duty to maintain the confidentiality of all information that is directly or indirectly acquired, created or disclosed to them in the course of providing treatment or care to children.¹⁴ This duty arises under common law, statute, and in equity, and it underpins the therapeutic relationship between the health professional and the child.¹⁴ This duty extends to other persons who come in contact with the information as part of the delivery of health care, including those staff members performing non-clinical activities. The duty to the child and their parent/guardian continues beyond the cessation of the professional relationship or beyond the death of the child.

Consent for the Release of Information

Consent to exchange information with referrers, key stakeholders and shared care arrangements must be clarified with the child (if the child has capacity) or their parent/guardian (if the child does not have capacity). Written consent using the CAMHS Consent for Exchange of Information form (MR510.07) must be given to release information to agencies and professionals external to CAHS, unless the disclosure of information is otherwise authorised or required by law.

When seeking consent to exchange information the CAMHS clinician must:

- Identify the agency or party that the information is to be exchanged with (their parent/guardian should be consulted about other agencies they deem relevant); and
- Discuss the nature and purpose of the information to be exchanged with the child and their parent/guardian (including what information is being sought from the other agency/party).

Consent or refusal of consent to exchange information must be documented in the medical record.

Implied Consent for the Release of Information

Where the child or their parent/guardian has consented to treatment from CAMHS, consent for sharing information with other CAHS health professionals for the purposes of treatment is implied. CAMHS do not need to obtain consent using the CAMHS Consent for Exchange of Information form (MR510.07) to liaise with other CAHS health professionals regarding treatment.

Implied consent does not arise where the child or their parent/guardian have refused to consent to treatment from CAMHS.

For further information, please refer to the [CAHS Sharing of Patient / Client Health Information Between CAHS Services policy](#).

Circumstances Requiring Disclosure of Health Information

Health information may be disclosed in the following circumstances:

- Where disclosure is required or permitted by the operation of the law; or
- Where the disclosure of information is reasonably necessary to lessen or prevent a serious risk to the life, health or safety of a person ([Health Services Act 2016](#) (WA) s 220(1)(i), [Health Services \(Information\) Regulations 2017](#) (WA) r 5(1)(a)). For further information with regards to risk assessment and confidentiality, please refer to the CAMHS [Risk Assessment and Management policy](#).

Circumstances where Consent is not Required

Emergency Psychiatric Treatment

Emergency psychiatric treatment can be provided to a child without informed consent (MHA s 203).

Emergency psychiatric treatment does not include ECT, psychosurgery or prohibited treatments such as deep sleep therapy, insulin coma therapy or insulin sub coma therapy (MHA s 202(2)).

The psychiatrist that provides emergency psychiatric treatment must provide a record of the provision of this treatment to the child or their parent/guardian, the Chief Psychiatrist and, if applicable, the Mentally Impaired Accused Review Board (MHA s 204(1)(b)).

This record must be recorded on [Form 9A – Record of Emergency Psychiatric Treatment](#).

Where emergency psychiatric treatment has been undertaken without consent, the rationale for treatment must be documented as soon as possible and must include:

- The nature of the risk and specific intervention;
- Any attempts to contact the parent/guardian (if relevant); and
- The outcome of the intervention.

Involuntary Mental Health Patients or the Mentally Impaired Accused

Psychiatric Treatment

Involuntary mental health patients or the mentally impaired accused can be provided treatment without informed consent (MHA s 178(1)). In these circumstances, the treating clinician should still have regard to the wishes of the child and their parent/guardian (MHA s 179(1)). Where possible, the treating clinician should still provide a clear explanation of the proposed treatment (MHA s 180(1)(a)) and give the child and their parent/guardian sufficient time to consider the proposed treatment (MHA ss 180(1)(b), 180(1)(c)).

The psychiatrist must ensure that a record of the following is documented in the medical record (MHA s 179(2)):

- Any wishes expressed by the child and their parent/guardian that the treating clinician was able to ascertain (MHA s 179(2)(a));
- Any other information that the treating clinician considered relevant in ascertaining the wishes of the child and their parent/guardian (MHA s 179(2)(b));

Urgent Non-Psychiatric Treatment

Where urgent non-psychiatric medical treatment is provided to involuntary mental health patients or the mentally impaired accused without informed consent, the person in charge of the authorised hospital must report the provision of non-psychiatric medical treatment to:

- The Chief Psychiatrist (MHA s 242(3)(a)); and
- The Mentally Impaired Accused Review Board (if applicable) (MHA s 242(3)(b)).

The report must be documented on [Form 9B – Report to the Chief Psychiatrist About Provision of Urgent Non-Psychiatric Treatment](#).

The provision of urgent non-psychiatric treatment is a notifiable event under the MHA (MHA s 242(5)). The person in charge of the authorised hospital must notify the child's parent/guardian, carer or nominated person (MHA s 139(1)), unless it is decided that the notification is not in the child's best interest (MHA s 142).

Documentation

Documentation of consent must include the following information:

- The child's name;
- Parent/guardian details of the child (name and relationship to the child);
- The matter they are consenting to (for example assessment, treatment or release of information);
- Determination as to capacity (in relation to the matter they are consenting to);

- Details about the information provided to the child and their parent/guardian;
- Key points of discussion including questions raised and responses;
- Any material risks;
- The signature of the child (where the child has capacity to consent) or their parent/guardian (where the child does not have capacity to consent);
- The name, designation and signature of the person obtaining consent;
- Whether an interpreter was required and provided; and
- The date of consent.

The person documenting the consent must also ensure that any other known outdated consent forms in the medical record are clearly marked as superseded.

Consent forms may be considered outdated if:

- A more recent consent form pertaining to the same matter is provided by the child or their parent/guardian; or
- Where circumstances requiring a [review](#) of consent have occurred.

For more information regarding consent documentation, please refer to [Appendix 1](#).

Compliance

Compliance with this guideline shall be audited annually with an approved auditing tool.

The focus of monitoring is to verify the informed consent process through auditing the use of an appropriate consent form and the recording of discussions between the clinician and the child or their parent/guardian in the child's medical record.¹²

Related CAHS internal policies, procedures and guidelines
CAHS Access and Use of Medical Records
CAHS Confidentiality, Disclosure and Transmission of Health Information Policy
CAHS Consent to Treatment Policy
CAHS Custodial Persons and Young Persons in Custody Policy
CAHS Sharing of Patient / Client Health Information Between CAHS Services
CAMHS Clinical Assessment Policy
CAMHS Clinical Engagement Policy
CAMHS Physical Health Assessment and Monitoring Policy
CAMHS Psychotropic Medication – Monitoring Adverse Physical Health Effects Policy
CAMHS Risk Assessment and Management Policy

References and related external legislation, policies, and guidelines
1. Carers Recognition Act 2004 (WA)
2. Children and Community Services Act 2004 (WA)
3. Clinicians' Practice Guide to the Mental Health Act 2014
4. Criminal Law (Mentally Impaired Accused) Act 1996 (WA)
5. Family Court Act 1997 (WA)
6. Family Law Act 1975 (Cth)
7. Guardianship and Administration Act 1990 (WA)
8. Guidelines on Privacy in the Private Health Sector
9. Health Services Act 2016 (WA)
10. Health Services (Information) Regulations 2017 (WA)
11. Mental Health Act 2014 (WA)
12. WA Health Consent to Treatment Policy

13. [WA Health Information Access, Use and Disclosure Policy](#)

14. [WA Health Patient Confidentiality Factsheet](#)

Useful resources (including related forms)

[Assessing Capacity to Consent in Minors: A Clinical Handbook](#)

[SSCD and MHA Forms](#)

This document can be made available in alternative formats on request.

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Healthy kids, healthy communities

Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Neonatology | Community Health | Mental Health | Perth Children's Hospital



Appendix 1: CAMHS Consent Documentation

Consent Type		Documentation	Application
Assessment	Mental Health Assessments	CAMHS Initial Assessment form	CAMHS-Wide
	Physical Health Assessments	CAMHS Physical Examination form	CAMHS-Wide
Treatment	Course of Treatment	Medical Record (Progress Notes)	CAMHS-Wide
		CAMHS Participation Agreement [MR816.18]	PCH CAMHS
	Medication	Consent for Medication Use [MR505.00/MR859.01]	CAMHS-Wide
Exchange Information		Consent to Exchange Information [MR510.07]	CAMHS-Wide
		Choice Summary*	Community CAMHS

* The Choice Summary only provides for consent to share that specific document.

